

STROKE PATIENT TRANSPORT PROTOCOL

Prehospital and Interfacility Transport - EMS & Stroke System of Care

Document Control - Document Number: STK-EMS-002 - Title: Stroke Patient Transport Protocol - Effective Date: [To be assigned upon approval] - Review Date: Annually - Approved By: Stroke Committee / EMS Medical Director

1. Purpose

To ensure timely, safe, and appropriate transport of patients with suspected acute stroke to the most appropriate stroke center while minimizing delays to reperfusion therapy.

2. Scope

This protocol applies to:

EMS agencies

Emergency Department staff

Stroke Team

Transfer Center

Critical Care Transport Teams

Air Medical Transport (when applicable)

3. Transport Eligibility

Patients shall be transported under the Stroke Alert pathway if they have:

Sudden onset of focal neurological deficit

Positive validated stroke screening tool (FAST, BE-FAST, CPSS, etc.)

Last Known Well (LKW) documented or estimated

Symptoms suggestive of acute ischemic or hemorrhagic stroke

4. Destination Selection

A. Primary Stroke Center (PSC)

Transport to the nearest PSC when:

Suspected acute stroke without evidence of large vessel occlusion (LVO)

Estimated transport time to Comprehensive Stroke Center (CSC) would cause significant treatment delay

Patient is a potential candidate for intravenous thrombolysis

B. Comprehensive Stroke Center (CSC)

Transport directly to a CSC when:

Positive LVO screening (FAST-ED, RACE, LAMS, VAN, etc.)

Suspected need for mecha0 0 rg BT5 206 C Q Q j T* ET Q15 q 1 0 0 1 40.01575 782.874 cm q 0 0 0 rg BT 1 0

Obtain IV access if it does not delay transport

Check blood glucose

Monitor airway, breathing, circulation, and neurological status

Avoid unnecessary procedures that delay arrival

6. Hospital Pre-Notification

EMS shall notify the receiving hospital before arrival and provide:

Estimated time of arrival (ETA)

Patient age and sex

Last Known Well

Stroke screening result

LVO score (if performed)

Blood glucose

Blood pressure

Current neurological deficits

Anticoagulant use (if known)

Airway status

7. Interfacility Transfer Protocol

Patients requiring higher-level stroke care shall be transferred without unnecessary delay.

Indications Include

Mechanical thrombectomy

Neurosurgical intervention

Neurocritical care

Advanced neuroimaging unavailable at referring hospital

Complex stroke management beyond local capability

Prior to Transfer, the Referring Hospital Shall

Stabilize airway, breathing, and circulation

Perform CT brain (and CTA if available)

Send imaging electronically whenever possible

Complete transfer documentation

Notify the receiving stroke team

Arrange the fastest appropriate transport

8. Mode of Transport

Ground Ambulance

Preferred when transport time is acceptable and rapid access is available.

Air Ambulance

Consider when:

Long transport distance

Time-sensitive thrombectomy candidate

Remote location

Ground transport would significantly delay definitive treatment

9. Monitoring During Transport

Monitor and document:

Blood pressure

Heart rate

Respiratory rate

Oxygen saturation

Glasgow Coma Scale (GCS)

Neurological status

Blood glucose (if indicated)

Maintain - SpO₂ ? 94% - Normoglycemia - Airway protection - Intravenous access

10. Documentation

Record:

Symptom onset / Last Known Well

Stroke screening tool results

LVO assessment

Vital signs

Blood glucose

Treatments provided

Time of EMS activation

Departure time

Arrival time

Hospital prenotification time

Any clinical deterioration during transport

11. Quality Indicators

Indicator	Target
EMS prenotification	? 95%
Last Known Well documented	? 95%
Blood glucose documented	? 95%
Scene time	? 15 minutes
Appropriate destination compliance	? 90%
Door-In-Door-Out (transfer hospitals)	? 60 minutes (goal)
Transfer imaging available before departure	? 95%

12. Staff Education

All EMS and transport personnel shall receive annual education on:

Stroke recognition

Stroke screening tools

LVO assessment

Destination decision-making

Safe transport practices

Hospital prenotification

Documentation requirements

Competency should be assessed annually.

References

American Heart Association / American Stroke Association. Guidelines for the Early Management of Patients With Acute Ischemic Stroke.

Regional EMS Stroke Destination Policy.

Hospital Stroke Program Policies.